



TEST REPORT

Name : Mr. SRIKANTH TURPU

Age/Gender : 42 Years / Male

BirthDate : 12-Jan-1984

Registration ID : 260120094346

Ref. By : Dr RACHANNA G

Sample Type : Whole Blood - EDTA

Registered on : 06-Jul-2026 07:55

Collected on : 06-Jul-2026 07:58

Released on : 06-Jul-2026 09:40

Printed on : 06-Jul-2026 11:55

Regn Centre : Dilsukhnagar - 12

COMPLETE BLOOD PICTURE (CBP)

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Haemoglobin <i>Method: Photometric Measurement</i>	: 13.7	gm/dL	13.0 - 17.0
Total RBC Count <i>Method: Coulter Principle</i>	: 5.0	millions/cumm	4.5 - 5.5
Packed Cell Volume / Hematocrit <i>Method: Automated, Calculated</i>	: 42.9	%	40.0 - 50.0
Mean Corpuscular Volume (MCV) <i>Method: Derived From RBC Histogram</i>	: 86.0	fL	83.0 - 101.0
Mean Corpuscular Hemoglobin (MCH) <i>Method: Calculated</i>	: 27.5	pg	27.0 - 32.0
Mean Corpuscular Hemoglobin Concentration (MCHC) <i>Method: Calculated</i>	: 32.0	gm/dL	31.5 - 34.5
RDW (CV) <i>Method: Automated, Derived From RBC Histogram</i>	: 13.7	%	11.6 - 14.0
Total Leucocytes Count (TLC/WBC) <i>Method: Coulter Principle</i>	: 6400	Cells/cumm	4000 - 10000
Differential count			
Neutrophils <i>Method: VCS Technology / Microscopy</i>	: 48	%	40 - 80
Lymphocytes <i>Method: VCS Technology / Microscopy</i>	: 37	%	20 - 40
Eosinophils <i>Method: VCS Technology / Microscopy</i>	: 7	%	1 - 6
Monocytes <i>Method: VCS Technology / Microscopy</i>	: 7	%	2 - 10
Basophils <i>Method: VCSn Technology and Microscopy</i>	: 1	%	0-2
Absolute Leucocyte Count			
Absolute Neutrophil Count <i>Method: Calculated</i>	: 3072	Cells/cumm	2000 - 7000
Absolute Lymphocyte Count <i>Method: Calculated</i>	: 2368	Cells/cumm	1000 - 3000



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COMPLETE BLOOD PICTURE (CBP)

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Absolute Eosinophil Count <i>Method: Calculated</i>	: 448	Cells/cumm	20 - 500
Absolute Monocyte Count <i>Method: Calculated</i>	: 448	Cells/cumm	200 - 1000
Platelet Count <i>Method: Coulter Principle and Microscopy of Leishman Stained Smear</i>	: 240000	Cells/cumm	150000 - 410000
Peripheral Smear			
RBC <i>Method: Microscopy of Leishman stained smear</i>	: Normocytic Normochromic		
WBC <i>Method: Microscopy of Leishman stained smear</i>	: Relative Eosinophilia		
Platelets <i>Method: Microscopy of Leishman stained smear</i>	: Adequate		



MC-8070

DR. HEMALATHA.S
MD PATHOLOGY

Registration No:76405



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ERYTHROCYTE SEDIMENTATION RATE (ESR)

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
ESR	: 5	mm/hour	0 - 15
Method: Automated - Modified Westergren			

Interpretation / Limitations:

- ESR is a nonspecific parameter, clinically useful in prognosis of disorders associated with an increased production of acute phase reaction proteins, immunoglobulins and plasma fibrinogen
- ESR is increased in acute and chronic infections, inflammatory disorders, neoplastic diseases, connective tissue disorders and pregnancy
- ESR is decreased in polycythemic disorders, sickle cell disease, hereditary spherocytosis and hypofibrinogenemia



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Ref. By : Dr RACHANNA G

Sample Type : Urine

Registered on : 06-Jul-2026 07:55

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CUE (COMPLETE URINE EXAMINATION)

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
<u>Physical Examination</u>			
Colour	: Pale Yellow		Pale Yellow
<i>Method : RGB Color Senser</i>			
Appearance	: Clear		Clear
Specific Gravity	: 1.025		1.000 - 1.030
<i>Method: Ion Exchange</i>			
<u>Chemical Examination</u>			
Reaction/PH	: 6.0		4.6 - 8
<i>Method : Double Indicator</i>			
Protein	: Negative		Negative
<i>Method: Protein error of indicators</i>			
Glucose	: Negative		Negative
<i>Method: GOD-POD</i>			
Urobilinogen	: Normal		Normal
<i>Method : Modified Ehrlich reaction</i>			
Bilirubin	: Negative		Negative
<i>Method: Diazonium</i>			
Ketones	: Negative		Negative
<i>Method : Nitroprusside reaction</i>			
Nitrites	: Negative		Negative
<i>Method: Diazonium</i>			
Leukocytes	: Negative		Negative
<i>Method: Diazonium</i>			
Blood	: Negative		Negative
<i>Method : Peroxidase activity</i>			
<u>Microscopic Examination</u>			
Pus Cells	: 1-2	cells / HPF	0 - 5
RBC	: Absent	cells / HPF	0 - 2
Epithelial Cells	: 1-2	cells / HPF	0 - 8
Casts	: Absent		Absent
Crystals	: Absent		Absent

Method : Reagent Strip Method, Microscopy

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* Suggested Clinical Correlation, If Necessary Kindly Discuss with signatory



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CUE (COMPLETE URINE EXAMINATION)



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Ref. By : Dr RACHANNA G

Sample Type : Fluoride Plasma

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Regn Centre : Dilsukhnagar - 12

FASTING PLASMA GLUCOSE

TEST NAME

Fasting Plasma Glucose

Method: Hexokinase

RESULT

: 102

UNIT

mg/dL

BIOLOGICAL REFERENCE INTERVAL

Normal : 70 - 99

Impaired Fasting Glucose : 100 - 125

Diabetes : ≥ 126

Interpretation / Limitations :

Impaired Fasting Glucose 100 - 125

Diabetes ≥ 126

- The diagnosis of Diabetes requires a fasting plasma glucose of ≥ 126 mg/dL and/or a random / 2 hr post glucose value of ≥ 200 mg/dL on at least 2 occasions.
- Very high glucose levels (>450 mg/dL in adults) may result in Diabetic Ketoacidosis & is considered critical.
- Reference : ADA (American Diabetes Association) guidelines – 2026 is adapted for the evaluation of diabetic status.



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GLYCATED HAEMOGLOBIN (HbA1C)

<u>TEST NAME</u>	<u>RESULT</u>	<u>UNIT</u>	<u>BIOLOGICAL REFERENCE INTERVAL</u>
Glycated Haemoglobin	: 5.6	%	< 5.7

Method: High performance liquid chromatography

Interpretation / Limitations:

Non Diabetic : < 5.7

Pre - Diabetic Range : 5.7 - 6.4

Diabetic Range : ≥ 6.5

- HbA1c is used to monitor glycemic control, evaluate the need to change therapy and predict the development of microvascular complications.
- The Glycemic target for non-pregnant adult females with diabetes is <7.0%.
- Lowering A1c value below or around 7.0% has been shown to reduce microvascular complications and macrovascular disease.
- Trends in HbA1C values, at interval of 3 months are a better indicator of Glycemic control than a single test.
- Glycated hemoglobin reflects the average of plasma glucose level during the previous 6-12 weeks.
- HbA1c result may not correlate with the blood glucose level in presence of Hemoglobin variants, conditions that affect red cell turnover, Systemic Inflammatory Diseases, Chronic renal failures, Liver diseases, recent blood transfusions and hypertriglyceridemia. Clinical Correlation is advised in interpretation of low Values.
- Reference Range is as per American Diabetes Association (ADA) 2026 Guidelines



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GLYCATED HAEMOGLOBIN (HbA1C)

3260120094346

Patient ID:

DOB:

Gender:

Physician:

Rack: 002

Position: 1

HbA1c:

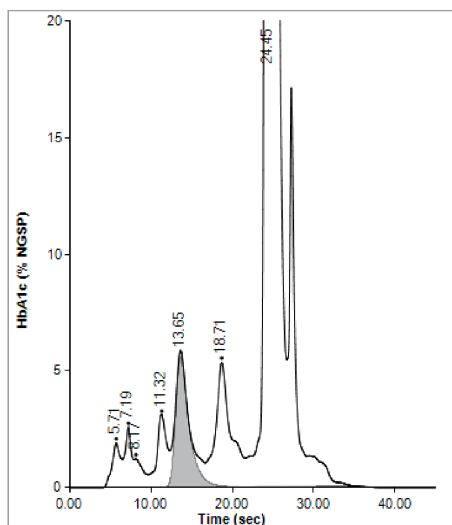
5.6 %

Run Date/Time:

06/07/2026 9:45:36

Note:

Comment:



Peak Name	RT (sec)	Area	Area%	Concentration (% NGSP)
A1a	5.71	1384.84	0.90	---
A1b	7.19	1516.20	0.99	---
F	8.17	794.51	0.52	---
LA1c	11.32	2958.03	1.92	---
HbA1c	13.65	6605.42	---	5.6
P3	18.71	7315.23	4.75	---
A0	24.45	133336.61	86.63	---
Total Area:		153911		

Status: **Released**



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Sample Type : Serum

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LIPID PROFILE

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Triglycerides <i>Method: Glycerol Phosphate Oxidase (GPO), Peroxidase (POD)</i>	: 88	mg/dL	Desirable level : < 150 Borderline : 150 - 199 High : 200 - 499 Very High : > / = 500
Total Cholesterol <i>Method: Cholesterol Oxidase Peroxidase (CHOD-POD)</i>	: 137	mg/dL	Desirable : < 200 Borderline high : 200 - 239 High : > / = 240
LDL Cholesterol <i>Method: Calculated</i>	: 74	mg/dL	< 100
HDL Cholesterol <i>Method: Enzymatic Immuno-inhibition</i>	: 45	mg/dL	Desirable level : > / = 60 Optimal : 40 - 59 Undesirable : < 40
VLDL Cholesterol <i>Method: Calculation</i>	: 18	mg/dL	< 30
Total Cholesterol / HDL Cholesterol Ratio <i>Method: Calculation</i>	: 3.0		3.3 - 4.4
LDL Cholesterol / HDL Cholesterol Ratio <i>Method: Calculation</i>	: 1.6		0.5 - 3.0

Interpretation / Limitations:

NCEP (ATP III) Recommendations	Triglycerides in mg/dL.	Total Cholesterol in mg/dL.	LDL Cholesterol in mg/dL.	HDL Cholesterol in mg/dL.
Desirable	< 150	< 200	-	> / = 60
Optimal	-	-	< 100	40 - 59
Near Optimal	-	-	100 - 129	-
Borderline High	150 - 199	200 - 239	130 - 159	-
High	200 - 499	> / = 240	160 - 189	-
Very High	> / = 500	-	> / = 190	-
Undesirable	-	-	-	< 40

Total Cholesterol / HDL Cholesterol Ratio:

Low Risk	3.3 - 4.4
Average Risk	4.5 - 7.1
Moderate Risk	7.2 - 11.0



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LIPID PROFILE

LDL Cholesterol / HDL Cholesterol Ratio

Desirabel Level	0.5 - 3.0
Borderline risk	> 3.0 - 6.0
High Risk	> 6.0

National Lipid Association Recommendations (NLA 2014)	Triglycerides in mg/dL.	Total Cholesterol in mg/dL.	LDL Cholesterol in mg/dL.
Optimal	< 150	< 200	< 100
Above Optimal	-	-	100 - 129
Borderline High	150 - 199	200 - 239	130 - 159
High	200 - 499	> = 240	160 - 189
Very High	> = 500	-	> = 190

- The panel of blood tests in Lipid profile serves as initial broad screening tool for abnormalities in lipids, can determine approximate risks for cardiovascular disease as also can point towards certain genetic diseases.
- Same person may have physiological variations in different measurements. Hence, three serial samples, ideally one week apart are recommended for Total Cholesterol, Triglycerides, HDL & LDL Cholesterol.
- All adults above the age of 20 years are recommended for screening for Atherosclerotic Cardiovascular Disease (ASCVD) risk factors especially lipid profile. This should be done earlier if there is family history of premature heart disease, dyslipidemia, obesity or other risk factors.



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SGPT / ALT (ALANINE TRANSAMINASE)

TEST NAME

RESULT

UNIT

BIOLOGICAL REFERENCE INTERVAL

SGPT/ALT

: 6

U/L

0 - 50

Method: IFCC without P-5-P

Interpretation / Limitations :

- Most sensitive test for Acute Hepatocellular injury (eg. viral, drug induced). It precedes the increase in serum bilirubin by approximately one week.



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Registration ID : **260120094346**
Ref. By : **Dr RACHANNA G**
Sample Type : **Serum**

Registered on : **06-Jul-2026 07:55**
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CREATININE

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Creatinine	: 0.6	mg/dL	0.7 - 1.2

Method: Jaffe Kinetic IDMS traceable

e-GFR (Glomerular Filtration Rate)	: 128.3	mL/min/1.73 m ²	>= 90
------------------------------------	----------------	----------------------------	-------

Method: Calculation, CKD EPI equation

Interpretation / Limitations :

- Serum Creatinine is useful in the diagnosis of renal insufficiency and is more specific and sensitive indicator of renal disease than serum Urea /BUN.
- Use of simultaneous Urea / BUN and creatinine levels provide more information in the diagnosis of renal insufficiency.
- Amount of Creatinine produced in the body is relatively constant (unlike Urea) and is primarily a function of muscle mass (Creatinine being a waste product formed in muscles.).
- Age, gender, Race, Muscle mass, Exercise, Certain Drugs, diet, Dehydration are amongst the physiological factors affecting serum creatinine concentration.
- GFR (Glomerular Filtration Rate) is generally considered the best index of overall kidney function. Repeated determination in conjunction with creatinine assay establish whether the patient has stable or progressive disease.
- The CKD-EPI equation is the most widely used IDMS traceable equations for estimating GFR in patients above 18 years of age. This equation includes variables for age and gender, and it may be observed that Kidney may be involved despite a serum creatinine concentration appearing to be within or just above the Biological Reference Interval. The results of e-GFR by CKD-EPI equation are normalized to 1.73 m² body surface area. CKD-EPI equation is not valid for individuals under 18 years of age.
- Estimates for GFR based on serum creatinine will be less accurate for patients at the extremes of muscle mass (such as frail elderly, critically ill, cancer patients) and also those with unusual diets, sudden acute renal failure, patients on dialysis and patients with severe liver disease.
- Confirmatory tests with exogenous measured GFR or directly measured creatinine clearance should be performed for such individuals.
- The influence of creatinine measurement imprecision at low creatinine concentrations (and hence high e-GFR) has a possible contribution to the variability at higher e-GFR values.



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THYROID PROFILE

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
T3 Total <i>Method: Chemiluminescence Immunoassay</i>	: 1.11	ng/mL	0.60 - 1.81
T4 Total <i>Method: Chemiluminescence Immunoassay</i>	: 8.30	µg/dL	3.2 - 12.6
TSH - Ultrasensitive <i>Method: Chemiluminescence Immuno Assay (CLIA)</i>	: 3.042	µIU/mL	0.55 - 4.78

Interpretation / Limitations :

- T3:** The levels of T3 helps in the diagnosis of T3 thyrotoxicosis and monitoring the course of hyperthyroidism. T3 is not recommended for diagnosis of hypothyroidism as decreased values have minimal clinical significance.
- Values below the lower limits can be caused by non-thyroidal illness, acute and chronic stress and hypothyroidism.
- T4:** Elevated level of T4 are seen in hyperthyroidism, pregnancy, euthyroid patients with increased serum thyroxine binding globulin.
- Decreased levels are noted in hypothyroidism, hypoproteinemia, euthyroid sick syndrome, decrease in thyroxine binding globulin.
- TSH:** TSH levels are increased in primary hypothyroidism, insufficient thyroid hormone replacement therapy, Hashimoto's thyroiditis, use of amphetamines, dopamine antagonists, iodine containing agents, lithium and iodine induced or deficiency goiter.
- Decreased levels of TSH may be seen in Grave's disease, toxic multinodular goitre, thyroiditis, excessive treatment with thyroid hormone replacement and central hypothyroidism.

----- End of Report -----



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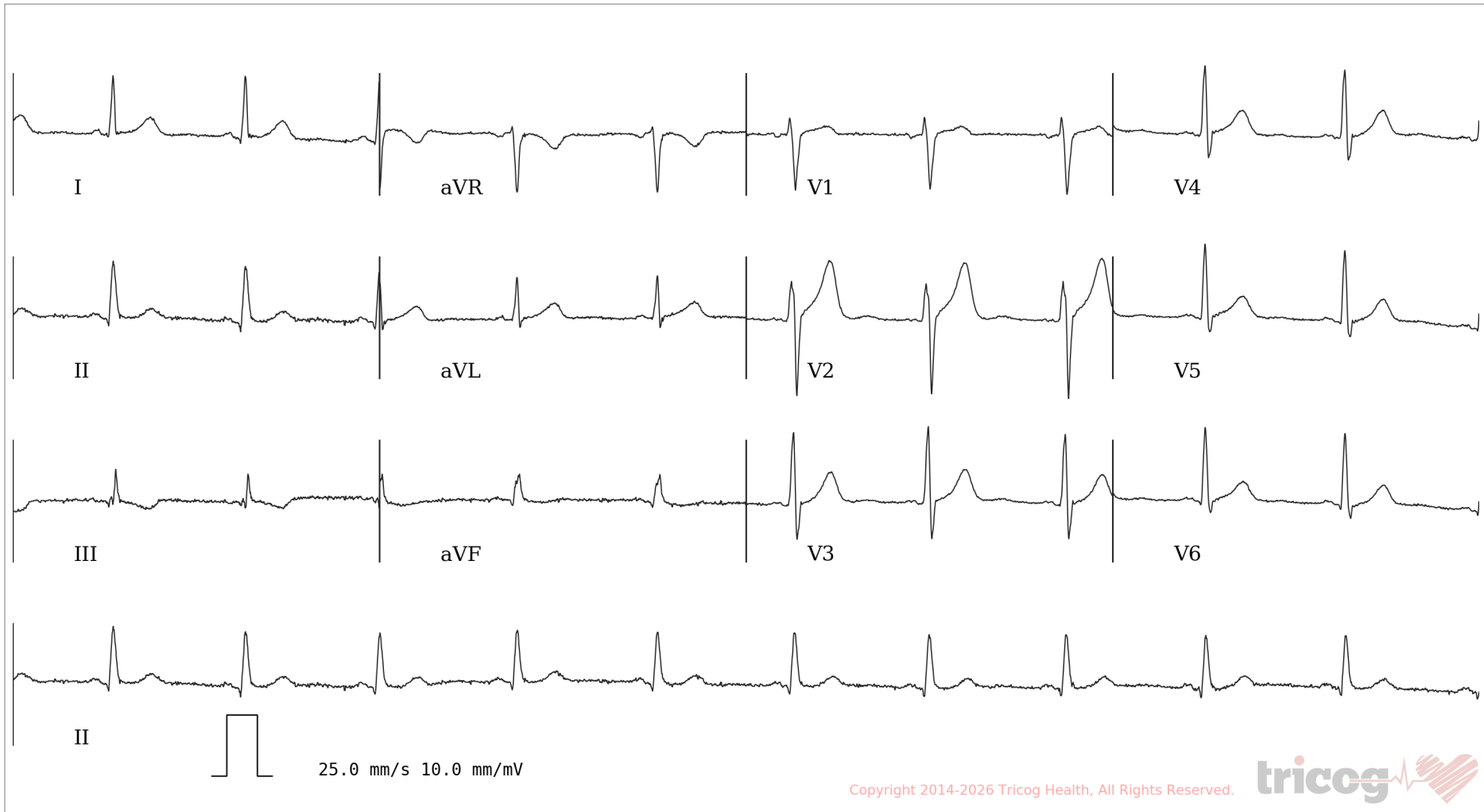
Patient Name: Mr. SRIKANTH TURPU

Age / Gender: 42/Male

Patient ID: 260120094346

Acquired At: 6 Jul 2026 8:29 AM

Reported At: 6 Jul 2026 8:31 AM



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AR: 67bpm

VR: 67bpm

QRSD: 100ms

QT: 396ms

QTcB: 418ms

PRI: 110ms

P-R-T: 23° 35° 3°

ECG Within Normal Limits: Normal sinus rhythm. Please correlate clinically.

REPORTED BY

DR. G. D. PRASAD
MD, DM

109026

Disclaimer: Analysis in this report is based on ECG alone and should only be used as an adjunct to clinical history, symptoms and results of other invasive and non-invasive tests and must be interpreted by a qualified medical professional.